Kakuri, Kaduna South, Kaduna State, Nigeria

The headline is heat, but the deaths that can be prevented in Kakuri this season are dry-season meningitis and cholera. Get diagnostics, drugs and a death count in place before January, and prepare heat measures for March to May without relying on fans or the grid.

IMPACT REGION	EXCESS HEAT DEATHS	WINDOW	DOMINANT HAZARD
Not resolved. Kaduna South LGA (402,731 people, 2006 census; 13 wards including Kakuri Gwari and Kakuri Hausa)	Nigeria 31,400 (15,400 in Dec–Feb). Population-scaled to Kaduna South: ~90, our arithmetic	6-month, Sep 2026 – Feb 2027	Meningitis (Dec–Jun, peak Feb–Apr) and dry-season cholera (onset Nov–Jan); heat peaks Mar–May; flooding unconfirmed for Kakuri

16 recommendations: 13 checked by verifiers, 3 not reached. 1 confirmed without correction, 8 corrected, 5 unverified, 2 contraindicated. **The funding target fell by more than a third in verification** ($95,000 → $45,000), because $50,000 sat in a programme outside the El Niño hazards. It is now listed separately. Exposure figure: Climate Impact Lab, *451,000 at Risk*, September 2026, pre-peer-review; its temperature–mortality functions were estimated with no African mortality data. **The ~90 figure for Kaduna South is a flat population scaling (402,731 ÷ 140.4**

* Suggested formulation: check with a health worker before sharing.

EXECUTIVE SUMMARY — AT A GLANCE



10.46°N 7.42°E, Nigeria · Natural Earth

- **The risk.** Dry-season meningitis and cholera kill here, not heat: Kaduna's 2014 cholera outbreak began in January and killed 54 (76 cases in Kaduna South), and Kaduna recorded 196 cholera deaths in 2022.
- **First action.** The Kaduna State PHC Board, Kaduna South LGA, and NCDC stock meningitis diagnostics and drugs at Kakuri's two hospitals by 15 December 2026, and confirm Men5CV coverage.
- **The money.** ~$25k floor, ~$45k target, ~$150k ceiling. The top placement is the Nigerian Red Cross Kaduna branch for ward volunteers, the death count, ORS, and zinc, $25,000.
- **Do not.** Do not wait for the epidemic threshold before preparing for meningitis: reactive vaccination prevented only about 3% of cases in Nigeria in 2015–17, because campaigns started six to nine weeks late.

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million × 31,400), our arithmetic, not an Impact Lab finding. Web search was unavailable for this run; agents fetched sources directly, and ReliefWeb and NCDC situation reports returned 403, so current 2026 Kaduna case counts could not be checked.

WHAT THIS PLACE SHOULD DO RANKED

Ranked by expected lives saved per unit of effort before the peak. Backing lenses are shown per action; an action several lenses reached independently is the stronger bet. The RACI chart below assigns each one.

1. Meningitis: put diagnostics and drugs in Kakuri's two hospitals before January, and find out whether Kaduna South ever received Men5CV

PARTIALLY CONFIRMED | EMERGENCY · PUBLIC HEALTH · SKEPTIC · HISTORIAN · FUNDER · ECONOMIST | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kaduna State Primary Health Care Board with Kaduna South LGA PHC department; NCDC for the vaccine request	15 Dec 2026	~~₦~~8–15m (lens estimate, no quote)	Deaths from late diagnosis and late vaccination

Stock ceftriaxone and lumbar-puncture and specimen-transport kits (with trans-isolate medium) at Dr Gwamna Awan General Hospital and Saint Gerard's Hospital, both in Kakuri. Pre-draft the International Coordinating Group vaccine request with serogroup confirmation built in, so it goes out the week Kaduna South passes the WHO epidemic threshold (more than 10 suspected cases per 100,000 a week; alert above 3). **Speed alone is not enough:** reactive campaigns in 2015–17 prevented only 3% of cases in Nigeria (Cooper et al. 2019), and modelling shows even a 15-day response averts about 35% (Delport et al. 2026), against a historical 75 days. That is why the case-management half of this action carries most of the weight.

* Suggested formulation: check with a health worker before sharing.

Corrections: no source shows Kaduna in the March 2024 Men5CV campaign (Jigawa and Yobe are named), so treat coverage as unknown until the Board confirms it. In 2017 Nigeria took nine weeks from threshold to campaign, with only about 41% of requested vaccine arriving. Ceftriaxone as WHO's empirical epidemic drug is from the 2014 WHO guideline, which could not be loaded in this run. Evidence comes from the meningitis belt itself, so transferability is good.

2. Dry-season cholera: pre-position ORS, zinc and a rehydration point in each Kakuri ward, and chlorinate supply, from November

PARTIALLY CONFIRMED | EMERGENCY · PUBLIC HEALTH · HISTORIAN · SKEPTIC · FUNDER | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kaduna State Primary Health Care Board; Kaduna State Water Corporation with the Kaduna State Environmental Protection Authority	Nov 2026 – Jan 2027	~₦10m (lens estimate)	Deaths from delayed rehydration in a slow-burn outbreak

Kaduna's 2014 outbreak began in January, in the dry season, and ran 33 weeks: 1,468 cases and 54 deaths across 7 LGAs, with Kaduna South at 76 cases. Only 1.4% of stool samples were tested (PMC5579440). Kaduna recorded 196 cholera deaths in 2022. In Nigeria's 2018 outbreak, waiting more than two days for care doubled the odds of death. ORS cuts diarrhoea deaths by about 69% (Munos et al. 2010, evidence graded low quality). Stock a modest supply of rapid tests with confirmatory culture, and run tankers with chlorinated water when the mains fail.

* Suggested formulation: check with a health worker before sharing.

Corrections: the Kaduna well-contamination study cited by one lens was about Salmonella in other LGAs and was dropped. Size the rapid-test stock to current Kaduna case counts, which this run could not retrieve. The agency's abbreviation (KEPA or KADSEPA) is unverified.

3. Start a weekly community death count by 1 December, to set a baseline before the hot season

PARTIALLY CONFIRMED | EMERGENCY · PUBLIC HEALTH | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Nigerian Red Cross Society, Kaduna branch, with ward heads and clergy; reported to the Kaduna State Ministry of Health	1 Dec 2026, weekly through May	Under ₦2m (lens estimate)	Deaths going uncounted, and therefore unfunded

Kaduna has no working vital registration at this level, so heat or outbreak deaths in Kakuri will not show up anywhere. Count burials weekly with a short verbal-autopsy form. Without a baseline from December, nobody can show in April whether deaths rose, or whether the response worked.

Precedent is thin: the Kano 2020 excess-death estimate is a preprint without baseline statistics, and child-mortality surveillance in Nigeria is new (Bauchi). No Kaduna precedent was found.

4. Ward-level outreach in Kakuri Gwari and Kakuri Hausa: vulnerable-household lists, neighbour buddies, and one message from pulpit, mosque and radio

PARTIALLY CONFIRMED | FRONTLINE · SKEPTIC · PUBLIC HEALTH | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Ward heads (Mai Unguwa) with church welfare committees (St Andrew Catholic, St Paul's Anglican, Christ Apostolic Church) and local mosque committees	Lists by 15 Dec; checks Feb–May	Volunteer time	Isolated people, under-5s and IDP households being missed

* Suggested formulation: check with a health worker before sharing.

List elderly people living alone, people with disabilities, displaced households and families with under-5s, and give each a neighbour who checks daily in hot spells and in meningitis season. Stiff neck with fever goes straight to Dr Gwamna Awan General Hospital or Saint Gerard's. Send one two-line message in Hausa and English: lid and treat stored water, start ORS early, never run a generator indoors, bring children with fever or stiff neck in the same day. Pastors read it on Sunday, imams on Friday, and it runs on Liberty Radio Hausa 103.1 and Capital Sounds 90.9 (KSMC). Recruit volunteers from inside each ward; visit in daylight and in pairs.*

Correction: "Kakuri Hausa" is a ward name rooted in settlement history, not a religious boundary. Kakuri is Christian-majority and no named mosque was found in sources, so map mosques on the ground before relying on that channel. Evidence is indirect: social isolation raised heat-death odds in Chicago 1995 (living alone OR 2.3), and a Kaduna polio package that included religious and traditional leaders cut non-compliant households 79% (2013–16), though the leaders' share cannot be separated.

5. Under-5 triage at Kakuri's primary health centres, February to May

CONFIRMED | PUBLIC HEALTH | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kaduna State Primary Health Care Board; referral to Yusuf Dantsoho Memorial Hospital and Barau Dikko Teaching Hospital	From 1 Feb 2027	Within routine PHC budgets	Heat and dehydration deaths in young children

Screen every child for dehydration, fever, danger signs and malnutrition (mid-upper arm circumference), and start ORS on the spot. In the one Sahelian cohort, under-5 deaths rose 3.7% per 1 °C (95% CI 0.3–7.3) against 2.6% for all ages (Diboulo et al. 2012, Nouna, Burkina Faso). Kaduna State's under-5 mortality is 187 per 1,000 and stunting 48.1% (DHS 2018, state level).*

* Suggested formulation: check with a health worker before sharing.

The evidence is an association from a rural Burkinabè cohort, used as a Sahelian analogue; Kakuri is urban and its rate is probably lower. No intervention trial exists.

6. Heat readiness for March–May: shaded, ventilated halls with water and ORS, and start a Red Cross heat protocol for 2027–28

PARTIALLY CONFIRMED | EMERGENCY · ECONOMIST · SKEPTIC | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kaduna State Emergency Management Agency (KADSEMA) with the Nigerian Red Cross Kaduna branch; church and mosque halls as sites	Halls named by 1 Mar 2027; protocol filed now	~~₦~~5m for water and ORS (lens estimate)	Heat illness in March–May, when Kaduna reaches 40 °C and above (record 43 °C)

Name day-time sites in church and mosque halls offering shade, cross-ventilation, drinking water, ORS and skin-wetting. If a generator runs, it stays outdoors and downwind, with a carbon monoxide alarm. In parallel, the Nigerian Red Cross starts a simplified heat Early Action Protocol with IFRC, tied to a NiMet heat alert, aiming for approval for the 2027–28 season.*

Corrections: a protocol cannot be approved in five months (Burkina Faso's took about a year), so it will not cover spring 2027. NiMet station-level heat alerts could not be verified. Cooling sites reach few people even in rich cities (only 42% of surveyed US residents knew their nearest one), so treat halls as a supplement to outreach, not the plan.

7. Ask NERC and Kaduna Electric's administrator to protect supply to hospital and PHC feeders during heat alerts

PARTIALLY CONFIRMED — RECAST | EMERGENCY · ECONOMIST | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kaduna State Government (advocacy) to NERC and the KAEDC interim Administrator	Request by Jan 2027	Load reallocation, near zero	Clinics losing power, cold chain and lighting in the hot season

* Suggested formulation: check with a health worker before sharing.

NERC dissolved Kaduna Electric's board on 10 August 2026 and appointed an Administrator; the utility carries 71.88% losses and about ₦456.5bn of market debt. The request has to go to NERC, which still regulates Kaduna, not to a state regulator.

Promises of protected supply from a utility in this condition are fragile. Clinics should keep their own solar or generator backup for vaccines and lighting.

Not reached by verifiers

UNVERIFIED

Pre-emptive oral cholera vaccine before the 2027 rains (Historian; NCDC to request from the ICG/Gavi stockpile). **Relay the Nigeria Hydrological Services Agency flood outlook and Shiroro dam-release notices through KADSEMA** (Historian; flooding was not confirmed for Kakuri itself). **Shade and water points at Kakuri market, 12:00–16:00, run by the traders' association and keke/okada union** (Frontline; union cooperation unverified). Treat these as leads, not recommendations.

* Suggested formulation: check with a health worker before sharing.

WHO DOES WHAT

RACI CHART

R = Responsible (does the work) · **A** = Accountable (one per action; signs off and answers for it) · **C** = Consulted (asked before) · **I** = Informed (told after). Built from the owners named in the verified actions. No organisation here has been contacted or has agreed to its role; every row is a proposal to put to them.

Action	State PHC Board	State Min. of Health	NCDC / NPHCDA	Kaduna South LGA	KADSEMA	Nigerian Red Cross (Kaduna)	State Water Corp. / EPA	Gwamna Awan GH & Saint Gerard's	Ward heads	Church & mosque leaders	Radio (Liberty, KSMC)	NiMet	NERC / KAEDC	IFRC
1. Meningitis diagnostics, drugs, vaccine request	**R**	**A**	**C**	**R**	**I**	**I**		**R**	**I**	**I**				
2. Dry-season cholera stock and safe water	**R**	**A**	**C**	**C**	**I**	**C**	**R**	**C**	**I**	**I**	**I**			
3. Weekly community death count	**C**	**A**	**I**	**C**	**I**	**R**		**C**	**R**	**R**				
4. Ward outreach, buddies, one message	**C**	**I**		**A**	**I**	**C**		**C**	**R**	**R**	**R**			
5. Under-5 triage at PHCs, Feb–May	**R**	**A**		**I**				**R**	**I**	**I**				
6. Heat halls and Red Cross heat protocol	**C**	**C**		**I**	**A**	**R**			**I**	**R**	**I**	**C**		**C**
7. Protected supply to hospital and PHC feeders	**I**	**R**			**C**			**I**					**A**	

Two gaps the chart makes visible. **The State Ministry of Health is accountable for five of seven actions**, so one meeting with the Commissioner's office unlocks most of the plan. **No row has a funded Responsible party yet** except where it sits inside a routine budget; the funding section below shows what is confirmed. For action 7 the State Ministry of Health is Responsible only for making the request; NERC is Accountable because only it can order the utility.

WHERE MONEY DOES THE MOST

FUNDING

The Philanthropist lens, reconciled against the ranked actions and verified. Placements are ranked by marginal lives saved per dollar. The envelope is bounded by what this place can spend before the peak, not by what it needs. Nothing pre-arranged covers this place: IFRC's 2026 cholera operation (MDRNG046, CHF 374,988, to 31 Dec) targets five LGAs in Borno only; Nigeria has no heat protocol; the World Bank's national cash-transfer programmes closed in 2025.

FLOOR

$25,000, conditional on the Red Cross confirming in writing that it will work in Kaduna South

TARGET

$45,000

CEILING

~$150,000, only once the held recipients are verified

LARGEST UNCERTAINTY

Whether any recipient actually operates inside Kakuri; the Red Cross branch sits in Kaduna North

* Suggested formulation: check with a health worker before sharing.

Floor = $25,000 Red Cross (50 volunteers × $60/month × 5 months = $15,000, plus ~$10,000 ORS and zinc; the $60 rate is an assumption). Target = $25,000 + $15,000 meningitis detection (held until a hospital recipient is confirmed) + $5,000 radio (held until a station is confirmed). **The lens's original $95,000 target included $50,000 for New Incentives**, which GiveWell rates at $1,500–6,000 per death averted and which operates in Kaduna State. It is strong value but funds routine childhood immunisation, not this season's hazards, and its Kaduna South coverage is unconfirmed, so it is shown below as a separate option outside the target.

RECIPIENT	USE	AMOUNT	MECHANISM	BASIS	VERDICT
Nigerian Red Cross Society, Kaduna branch (6 North Road, Kaduna North), via national HQ	Actions 2, 3 and 4: ward volunteers, death count, ORS and zinc	$25,000	Restricted grant	Unit costs are lens estimates, not quotes; ORS cuts diarrhoea deaths ~69% (Munos 2010, low-quality evidence)	Partially confirmed: branch exists; no record of spending at this scale; no duplication, since the cholera DREF covers Borno only
Dr Gwamna Awan General Hospital or Saint Gerard's Hospital (to confirm)	Action 1: specimen transport and ceftriaxone	$15,000 (held)	Pre-positioned stock	Meningitis case fatality 4.5% in the belt (WHO, 2025); vaccine itself is supplied free through the ICG stockpile, so donors should not buy it	Unverified recipient
Liberty Radio Hausa or KSMC (to confirm)	Action 4: the two-line message on air	$5,000 (held)	Direct grant	No published cost-effectiveness figure	Unverified
Outside the target: New Incentives	Routine childhood immunisation incentives, Kaduna State	$50,000 (optional)	Unrestricted gift; a Kaduna request cannot be enforced	GiveWell: $1,500–6,000 per death averted, ~$18 per child enrolled	Partially confirmed; off-mechanism for El Niño

The funder's one thing: the headline heat number is not what this money buys down. Every placement with a known cost per death targets the diseases that kill Kakuri's children and young adults in the dry season. Fund those, say so plainly, and don't put money into heat messaging just because the headline says heat.

* Suggested formulation: check with a health worker before sharing.

DO NOT DO THIS CONTRAINDICATED

Advice that circulates for this hazard and would cost lives here.

"Use a fan" in the March–May heat *

CONTRAINDICATED

Meade et al. (*Lancet Planetary Health* 2024) conclude that health agencies should advise against fan use above 35 °C, especially for people who sweat less, such as adults over 65. Kaduna's hot season regularly exceeds that. Fans also need power that Kakuri often doesn't have.

Instead: shade, cross-ventilation at night, wetting skin and clothing, drinking safe water, and moving to a cooler place in the hottest hours.

Running a generator indoors or in a closed compound during outages *

CONTRAINDICATED

Carbon monoxide builds up in closed spaces, and generators run hardest exactly when the grid fails during heat. No Nigerian death count was verifiable in this run, but the mechanism is not in doubt.

Instead: generators outdoors and downwind of doors and windows; carbon monoxide alarms in clinics and any hall used as a heat site.

Waiting for the epidemic threshold before preparing for meningitis

CONTRAINDICATED

Reactive vaccination after the threshold prevented about 3% of cases in Nigeria in 2015–17 (Cooper et al. 2019), because campaigns started six to nine weeks too late.

* Suggested formulation: check with a health worker before sharing.

Instead: pre-position drugs and diagnostic kits, and pre-draft the vaccine request (action 1).

Relying on the IFRC cholera fund MDRNG046 for Kakuri

CONTRAINDICATED AS PROPOSED

The operation's document names five LGAs in Borno and never mentions Kaduna. Planning on that money would leave Kakuri with nothing.

Instead: the Kaduna branch asks national HQ to raise Kaduna in cholera coordination, and seeks a separate allocation if Kaduna reports cases.

"Drink more water" as the main heat message, and treating ward names as religious boundaries*

CONTRAINDICATED

Heat and cholera both peak when water is scarce, so "drink more" without "drink safe" can spread disease. And "Kakuri Hausa" is a ward name, not a Muslim enclave: assigning faith channels by ward name would miss people in both wards.

Instead: "treat or boil stored water, keep it covered, start ORS early", delivered through every faith venue that exists in each ward.

WHAT HAS HAPPENED HERE BEFORE — THE RECORD

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
1996 (La Niña dry season)	Meningitis belt epidemic	~250,000 cases, ~25,000 deaths across the belt	Led to MenAfriVac (from 2010)
2000	Kaduna riots; protesters marched from Kakuri	1,295 official deaths; HRW estimate 2,000–5,000	Residential separation by religion deepened (preprint evidence)
2009 (La Niña dry season)	Meningitis, Nigeria	9,086 cases, 562 deaths in 217 LGAs	~4.5 million vaccinated reactively in nine states, Kaduna among them

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
2014	Cholera, Kaduna State, starting in January	1,468 cases, 54 deaths; Kaduna South 76 cases	Not documented; only 1.4% of samples were tested
2021–22	Cholera, Nigeria; Kaduna a hotspot	Kaduna 196 deaths in 2022	Not documented
2022	Kaduna River flooding	Damage recorded; Kakuri-specific impact unconfirmed	A federal flood-control project exists; floods recurred
2023–24	Meningitis serogroup C, seven northern states	1,742 cases, 153 deaths	Men5CV campaign, March 2024 (Kaduna's inclusion unconfirmed)
Aug 2026	NERC dissolves Kaduna Electric's board	Utility at 71.88% losses	Administrator appointed

The recurrence nobody has noticed: Kaduna's worst meningitis years (1996, 2009) came in La Niña dry seasons, not El Niño ones (NOAA index, DJF −0.9 both years). The clock that kills here is the dry season, whatever ENSO is doing, and every reactive campaign so far arrived after the deaths. Kakuri also lost the employer clinics and shared water of its textile-mill years, so it meets each season with less than it had in the 1990s.

WHERE THE EXPERTS DISAGREE — UNRESOLVED

- **Prepare heat-relief halls now** (Emergency manager) versus **do nothing new for heat in the December–February window** (Skeptic). Resolved in part: halls are planned for March–May, with no fans or indoor generators. What would settle it: Kakuri death counts by month from action 3.
- **Cholera follows the rains** (Historian, 2010 national outbreak) versus **Kaduna's outbreaks start in the dry season** (Emergency manager, 2014 Kaduna). Both are documented; the plan covers both, with dry-season stock first.
- **The 31,400 figure guides action here** versus **it is a model artefact aimed at the wrong months** (Skeptic, Public health). This report uses it only to rank Nigeria as a priority, not to count lives in Kakuri.

* Suggested formulation: check with a health worker before sharing.

WHAT NOBODY COVERED

BLIND SPOT

No lens could see **what is happening in Kaduna right now**. NCDC situation reports and ReliefWeb were blocked, so current cholera and meningitis counts, and whether Kaduna South received Men5CV, are unknown. Those two facts could reorder the whole list: a live cholera outbreak would move action 2 to the top, and confirmed Men5CV coverage would shrink action 1 to case management. Nobody has contacted the people named in the RACI chart, and nobody mapped the Muslim community's venues in Kakuri. The first job is a phone call to the State Primary Health Care Board.

HOW TO READ THIS

METHOD

The panel is author-built. Seven simulated expert lenses were used to generate these recommendations. Where they agree, that is a strong hypothesis, not a consensus of the field. Every recommendation was then independently checked for evidence, transferability to this location, and feasibility before the peak; the verdict tags carry the result.

Mortality figures are resolved at Climate Impact Lab impact-region scale, roughly 300,000 people. They are not neighbourhood figures. The El Niño projection covers *heat only*; drought, flood, fire, smoke, and crop failure are additional to it and are carried here by the historical record rather than by a published number.

HOW THIS BRIEFING WAS PRODUCED

METHODOLOGY

- **Scope.** The location was taken as given: Kakuri, Kaduna South LGA, Kaduna State, Nigeria, which spans two wards (Kakuri Gwari and Kakuri Hausa). Climate Impact Lab impact region, as the report records it: not resolved. Mortality figures exist only at that scale (impact regions average roughly 300,000 people); sub-city detail is used for hazard exposure and action design, not for death counts.
- **Exposure.** Climate Impact Lab, *451,000 at Risk* (September 2026, pre-peer-review): ECMWF SEAS5 seasonal forecasts (51-member ensemble) applied to temperature–mortality relationships across 24,378 regions, against a 1996–2025 baseline, for June 2026 – February 2027. It covers heat only. It is never added to the Impact Map's long-run projections or quoted from the map's GDP-denominated mortality. Where heat is not the dominant hazard, the historical record carries the hazard instead of a borrowed number.
- **Expert panel.** Seven simulated expert lenses (emergency manager, public health scientist, frontline responder, skeptic, economist, historian, and philanthropist) each researched this place independently and in parallel, from web sources, to a fixed brief.

* Suggested formulation: check with a health worker before sharing.

- **Reconciliation.** Duplicate actions were merged and backing lenses recorded. Convergence was tested for shared bias (whether the evidence behind an agreed action came from a setting like this one) before it counted in an action's favour. Contradictions were kept, not resolved by dropping a side. Actions were ranked by expected lives saved per unit of effort before the peak, each was given a named payer and trigger or marked unfunded, and the funding placements were reconciled against the ranked list. The RACI chart was added at Angus's request and built from the owners named in the verified actions.
- **Verification.** Five verifier agents checked 13 of the 16 recommendations on three tests: evidence (the primary source and its effect size), transferability (whether that evidence came from somewhere like here), and feasibility (whether it can be done here before the peak). Financial actions were also checked for availability in this jurisdiction, a named payer, a trigger threshold, and whether they already exist. The funding envelope was checked for real recipients, published unit costs, and duplication, then re-summed. Three leads were not reached and are marked unverified.
- **Verdicts.** *Confirmed*: evidence, transferability, and feasibility all hold. *Partially confirmed*: holds after the corrections shown. *Unverified*: could not be checked to a primary source. *Contraindicated*: would do harm or waste the window here.
- **Adaptation Inventory.** Not consulted: web search was exhausted for this session, and in earlier runs the inventory's download sat behind a sign-up form. No action is marked as having or lacking a precedent there.
- **Health wording.** Passages where a wording error could hurt someone (danger signs and when to go to the clinic, ORS and water treatment, cooling and fans, and generators and carbon monoxide) are marked "* Suggested formulation: check with a health worker before sharing." They were checked against published sources, not by a health professional. They are suggestions, not medical advice, and anyone passing them on should confirm them with a doctor, pharmacist, nurse, or community health worker first. The marker was added on 2 November 2026.
- **Rules.** No cost is derived by dividing a published total, and no local figure is invented; where data is thin, the report says so and shows the containing geography's number with its arithmetic. Every source is listed below with its verification status.
- **Limits.** The panel and verifiers are AI research agents working from web-available sources, not human experts, and no one in this place was interviewed. Run-specific gaps: web search unavailable (agents fetched pages directly); ReliefWeb, NCDC situation reports, CERF and several Nigerian news sites returned 403; the impact region is unresolved; current 2026 Kaduna case counts, Men5CV coverage in Kaduna South, NiMet heat alerts, and the abbreviation of the state environmental agency were not verified; lens cost figures in naira are estimates without quotes.

* Suggested formulation: check with a health worker before sharing.

References · Meningitis: Cooper et al. 2019, Delport et al. 2026 (doi 10.1186/s44263-025-00239-6), Nigeria serogroup C 2023–24, Nigeria 2017 ICG response, 2009 outbreak (confirmed, corrected) · Cholera: Kaduna 2014, Nigeria 2022–24, care-seeking delay, Munos et al. 2010, IFRC MDRNG046 (confirmed; Borno only) · Heat: Diboulo et al. 2012, Meade et al. 2024, Kaduna climate (confirmed) · Place: Kakuri, Kaduna South, 2000 riots, radio stations, Haliru & Shotorbani 2025 preprint (doi 10.21203/rs.3.rs-6883392/v1) (confirmed, corrected) · Outreach evidence: Semenza 1996, Kaduna polio 2013–16 · Power: NERC interim order on KAEDC · Funding: Nigerian Red Cross branches, GiveWell on New Incentives, MSF Nigeria, World Bank projects P176935, P174114, P504693 (confirmed) · ENSO: NOAA ONI
Exposure data · Climate Impact Lab, 451,000 at Risk · Impact Map · Adaptation Inventory
Generated 2 October 2026 for Kakuri, Kaduna South, Kaduna State, Nigeria. El Niño Ready is volunteer-run and is not affiliated with Climate Week NYC, The Climate Group, or Columbia University.

* Suggested formulation: check with a health worker before sharing.